

Pemetrexed + cisplatin

Lung · NCCP 00317 · version 7

Your medicines

Pemetrexed · Cisplatin

Your treatment schedule

Every 21 days · See page 2 for the treatment phases.

A picture of side effects

Tell your team about new or worsening symptoms. The same symptom can have more than one cause. Numbers connect each symptom box to its location.

Pemetrexed · marrow / mouth / skin / kidney

Cisplatin · kidney / hearing / nerves / nausea

1 Blood / infection

Pemetrexed / cisplatin

Low blood counts can cause infection, bruising or anaemia. Fever or sudden illness needs prompt oncology advice.

3 Hearing / tinnitus

Cisplatin

Report new ringing in the ears or hearing change; cisplatin-related hearing damage can persist.

5 Nausea / vomiting

Cisplatin

Cisplatin has high emetogenic potential. Take the prescribed anti-sickness medicines and call if you cannot keep fluids down.

7 Skin

Pemetrexed

Rash can occur; use the prescribed corticosteroid prophylaxis and report severe skin change.

2 Kidneys / hydration

Pemetrexed / cisplatin

Kidney function is important for both medicines. Cisplatin requires hydration and electrolyte monitoring.

4 Nerves

Cisplatin

New numbness, tingling or difficulty with fine movement/walking should be reported before the next dose.

6 Mouth / bowel

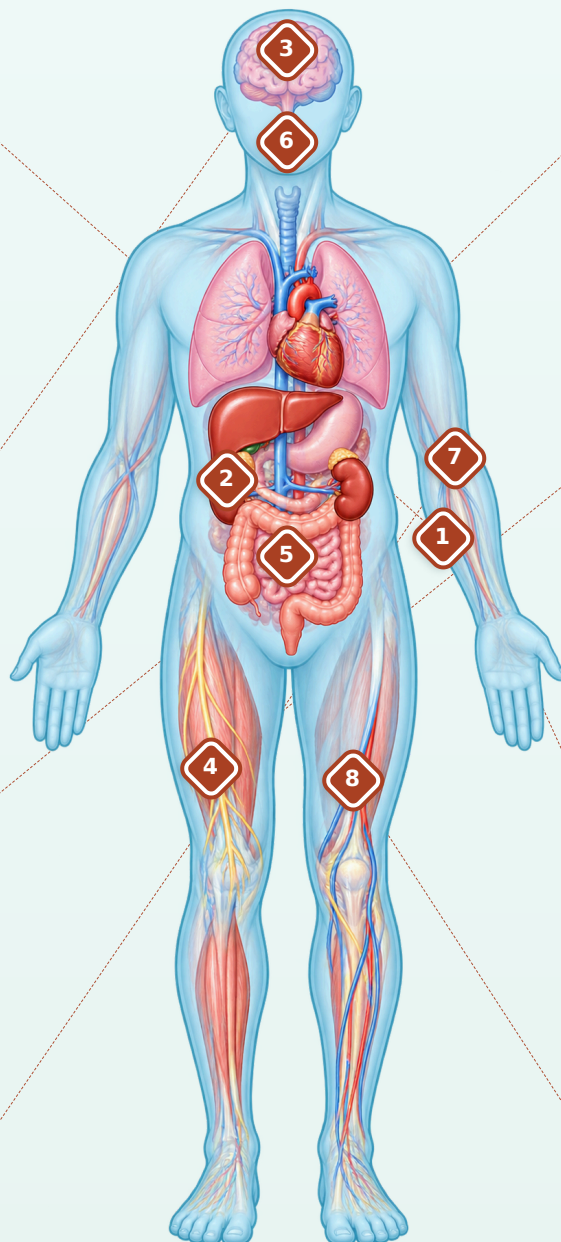
Pemetrexed

Mouth soreness or significant diarrhoea can occur and may require treatment review.

8 Electrolytes / whole-body symptoms

Cisplatin

Electrolytes can fall. Report marked weakness, cramps, palpitations or severe dizziness.



This map shows selected effects to discuss; it does not show their likelihood or every possible side effect. Blood-count and whole-body effects are marked schematically.

Selected symptom patterns for discussion. Tell your oncology team about any new or worsening symptom. See page 2 for medicine-specific effects, monitoring and the portal QR.

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TREATMENT JOURNEY

Day 1 Pemetrexed 500 mg/m ² IV, followed by cisplatin 75 mg/m ² IV with pre- and post-hydration.	Every 21 days Treatment repeats every 21 days.	Mesothelioma Usually up to 4-6 cycles unless progression or unacceptable toxicity occurs.	NSCLC Usually 4-6 cycles, then maintenance pemetrexed (NCCP 00222) where appropriate.
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IMPORTANT MEDICINE-SPECIFIC EFFECTS

Pemetrexed - Low blood counts can increase infection, anaemia or bruising risk. - Mouth/gut inflammation and skin rash can occur. - Kidney function can deteriorate and is monitored.	Cisplatin - Kidney injury and low magnesium/potassium can occur. - Hearing loss or ringing in the ears may be permanent. - Numbness/tingling and marked nausea/vomiting can occur.
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Other / shared treatment effects: The anatomy page highlights important symptom patterns for this exact regimen, but it cannot show every possible effect. Fatigue, nausea, appetite change, mouth/bowel symptoms or blood-count changes may occur depending on the medicines used. Report new or worsening symptoms even if they are not listed here.

Monitoring / medicines - Folic acid and vitamin B12 support are required with pemetrexed. - Cisplatin requires hydration and electrolyte management according to local policy. - Blood count, kidney and liver profile are checked before each cycle.	Bring / track - Audiology may be considered where clinically indicated. - Use the regimen-specific high-emetogenic-risk antiemetic prescription.
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WHEN TO CALL - DO NOT WAIT FOR THE NEXT APPOINTMENT
Contact oncology urgently for fever, inability to keep fluids down, marked reduction in urine, severe weakness, new hearing change, severe numbness/weakness, significant bleeding or rapidly worsening symptoms. For a medical emergency call 112/999.

Consent discussion support

Use the approved consent process to document treatment aim, expected benefit, material risks, alternatives, questions and the patient's decision.

Scan to open the regimen-specific patient portal

Regimen information only; this QR contains no patient details.

<https://sactcheck.com/patient/00317/>

Official NCCP 00317 v7 · SACTCheck v0.81.2 · patient information prototype for clinical review.

